

ADHCRI Investment Business Case



Document Control

- 1. Executive Summary
 - 1.1 Why ASEAN? - The Business Need
 - 1.2 Why Now?
 - 1.3 The Cost of Inaction
 - 1.4 Investment Vision
 - 1.5 Strategic Objectives
- 2. Evidence Supporting the Investment
 - 2.1 Current State Assessment
 - Introduction
 - Current State Assessment Matrix
 - Key Findings
 - 2.2 ASEAN Digital Health Investment Readiness Framework (ADHIRF)
 - Framework
 - Executive Insight
- THE BUSINESS CASE
 - 3.1 Proposed Business Solution
 - 3.2 Solution Overview
 - 3.2 Solution Capabilities
 - Executive Insight
 - 3.3 Financial Investment
 - Executive Takeaway
 - Executive Takeaway
 - 3.5 Sensitivity Analysis
- 4. Investment Risks and Mitigation
 - Executive Takeaway
- 5. Investment Recommendation and Funding Decision
 - Investment Recommendation
 - Final Investment Position
 - Executive Takeaway

Investment Required	Projected ROI	Countries	SDG Alignment	Implementation
US\$560M 5-Year CAPEX & OPEX	133.7%	10 ASEAN Member States	3 • 10 • 17	5-Year Phased Rollout (Tier A → B → C)

ADHCRI is a regional digital health ecosystem designed to improve cross-border health intelligence, strengthen climate resilience, and enable data-driven public health decision-making across all 10 ASEAN member states.

Document Control

Document	ADHCRI Investment Business Case
Project Title	Project Hygieia

Prepared by	Pod Nova (BA 10Alytics Team)
Audience	ASEAN Executive Board / Funding Partners
Version	1.0
Date	July, 2026

1. Executive Summary

The **ASEAN Digital Health & Climate Resilience Initiative (ADHCRI)** is a proposed regional programme aimed at improving healthcare resilience through better cross-border collaboration, data-driven decision-making, and stronger digital health capabilities across the ten ASEAN member states.

This Business Case presents the investment rationale, business requirements, and implementation strategy needed to support the successful rollout of the ADHCRI platform. It demonstrates the project's economic viability through a five-year cost-benefit analysis and outlines how the initiative can deliver measurable health, social, and financial benefits across the region.

The document also assesses the current readiness of ASEAN member states, identifies key capability gaps, and recommends practical transition strategies to support phased implementation. Together with the supporting BRD, Gap Analysis, and Agile Jira Backlog, it provides a clear and investment-ready roadmap for delivering a scalable and sustainable digital health ecosystem.

1.1 Why ASEAN? - The Business Need

The ASEAN region is home to more than **660 million people** across ten diverse member states with varying levels of healthcare maturity, digital infrastructure, and economic development. While individual countries have made significant progress in improving healthcare outcomes, growing cross-border disease transmission, climate-related health risks, and increasing pressure on public health systems require a coordinated regional response rather than isolated national initiatives.

Public health challenges no longer stop at national borders. Infectious diseases, environmental hazards, and humanitarian emergencies increasingly affect multiple countries simultaneously, highlighting the need for a shared digital health ecosystem capable of supporting regional collaboration, timely information sharing, and evidence-based public health decision-making.

Investment in the ASEAN Digital Health & Climate Resilience Initiative (ADHCRI) represents an opportunity to strengthen regional health resilience, improve collaboration among member states, and establish a sustainable foundation for future public health preparedness.

1.2 Why Now?

The need for regional investment in digital public health has become increasingly urgent. Rapid urbanization, increased cross-border mobility, climate-related disruptions, and the growing frequency of infectious disease outbreaks have exposed limitations in the region's existing public health infrastructure. While individual ASEAN member states continue to strengthen their national healthcare systems, fragmented digital health capabilities reduce the region's collective ability to anticipate, coordinate, and respond effectively to emerging public health threats.

At the same time, widening disparities in digital infrastructure, healthcare funding, and technological readiness increase the risk of unequal health outcomes across member states. Delaying investment will not only increase future implementation costs but also reduce ASEAN's ability to protect vulnerable populations, maintain healthcare continuity during climate-related disruptions, and achieve its long-term sustainable development objectives.

Investment at this stage provides an opportunity to establish a coordinated regional foundation before these challenges become more complex and significantly more expensive to address.

1.3 The Cost of Inaction

Failure to invest in a coordinated regional digital health ecosystem will expose ASEAN to increasing operational, financial, and public health risks. As disease outbreaks become more frequent and climate-related disruptions intensify, existing fragmented systems will continue to limit timely information sharing, delay emergency response, and reduce the effectiveness of regional healthcare coordination.

Without ADHCRI, ASEAN is likely to experience:

- Increased healthcare expenditure resulting from delayed outbreak detection and response.
- Greater economic losses caused by prolonged public health emergencies and workforce disruption.

- Continued inequalities in healthcare access across digitally underserved communities.
- Higher implementation costs as ageing digital infrastructure becomes more difficult and expensive to modernize.
- Reduced regional preparedness for future pandemics, climate-related disasters, and emerging public health threats.

Investing today is therefore not only a strategic opportunity but also a proactive measure to reduce future financial exposure, strengthen regional resilience, and protect long-term public health outcomes.

Having established the strategic need for investment, the following section presents the long-term vision and strategic objectives that will guide the successful implementation of the ADHCRI programme

1.4 Investment Vision

The investment vision for ADHCRI is to establish a secure, interoperable, and climate-resilient digital public health ecosystem that enables ASEAN member states to share health intelligence, predict disease outbreaks, improve healthcare decision-making, and ensure continuity of healthcare services during public health emergencies and climate-related disruptions.

By strengthening regional collaboration and enabling timely access to trusted public health information, ADHCRI will provide governments and public health agencies with the capabilities required to make faster, evidence-based decisions that improve health outcomes, optimize resource allocation, and strengthen regional resilience.

1.5 Strategic Objectives

ID	Strategic Objective
SO-01	Enable faster, evidence-based public health decision-making through trusted cross-border health intelligence.
SO-02	Improve interoperability by establishing secure and standardized health data exchange across all ASEAN member states.
SO-03	Strengthen regional resilience through earlier disease detection, coordinated emergency response, and climate-informed public health planning.
SO-04	Reduce healthcare inequalities by supporting phased implementation across countries with different levels of digital maturity and infrastructure readiness.
SO-05	Deliver measurable economic and societal value through improved operational efficiency, reduced outbreak costs, and sustainable long-term investment returns.

2. Evidence Supporting the Investment

2.1 Current State Assessment

Introduction

The assessment of ASEAN's current public health landscape reveals significant disparities in digital health maturity, infrastructure readiness, healthcare funding, and cross-border collaboration. While several member states have invested in strengthening national healthcare systems, the absence of a coordinated regional digital health ecosystem continues to limit ASEAN's collective ability to anticipate, detect, and respond effectively to emerging public health threats.

These findings demonstrate that the challenge is not the availability of health information, but rather the ability to integrate, govern, and transform that information into timely decisions that improve regional public health outcomes.

Current State Assessment Matrix

Challenge / Capability Area	Current State	Investment Implication
-----------------------------	---------------	------------------------

Healthcare Funding Inequality	Healthcare and digital health investment varies significantly across ASEAN member states. Lower-resourced countries have limited capacity to modernize infrastructure, maintain digital systems, and provide equitable access to healthcare.	Regional and external funding is required to reduce implementation disparities and prevent lower-readiness countries from being excluded from the programme.
Rising Climate-Driven Disease Burden	Climate change is increasing exposure to infectious and vector-borne diseases, including tuberculosis, dengue, and malaria. Existing surveillance and reporting processes remain fragmented and largely reactive.	Investment is required in regional disease intelligence, predictive surveillance, and coordinated response capabilities to reduce outbreak impact and associated economic losses.
Food Security and Child Health	Climate-related disruption to agriculture contributes to food insecurity and undernutrition, placing children and vulnerable populations at greater risk of preventable illness.	Integrated health and climate information is needed to support targeted interventions, early identification of vulnerable communities, and more effective allocation of public health resources.
Fragile Digital Health Infrastructure	Health information is distributed across disconnected legacy systems. Rural facilities also face unreliable electricity, weak internet connectivity, and limited access to modern technology.	A phased, interoperable, and climate-resilient approach is required to support data exchange and continuity of healthcare services across different infrastructure conditions.
Cybersecurity and Data Protection	Fragmented systems and inconsistent security standards increase exposure to ransomware, unauthorized access, data loss, and misuse of sensitive health information.	Investment must include strong cross-border data governance, access controls, auditability, privacy protection, and consistent regional security requirements.

Internet & Power Infrastructure	Connectivity and electricity reliability vary significantly, particularly in rural areas.	Supports phased deployment and resilient architecture.
Workforce & Digital Readiness	Digital literacy and organizational readiness differ across countries.	Requires tailored change management, training, and adoption strategies.
Cross-Border Collaboration & Governance	Policies, standards, and data-sharing practices differ across member states.	Requires harmonized governance to enable trusted regional collaboration.

Key Findings

- The five challenges are interconnected and cannot be addressed effectively through isolated national initiatives.
- Funding inequality creates different levels of readiness and requires a phased, country-sensitive investment approach.
- Climate-driven health risks increase the urgency for earlier detection and coordinated regional decision-making.
- Infrastructure limitations threaten service continuity, particularly in rural and underserved communities.
- Cybersecurity and data protection must be treated as core investment requirements, not later-stage technical additions.

2.2 ASEAN Digital Health Investment Readiness Framework (ADHIRF)

To support investment prioritization and phased implementation, this Business Case introduces the **ASEAN Digital Health Investment Readiness Framework (ADHIRF)**.

The framework evaluates the critical business capabilities that influence the successful deployment of ADHCRI across member states. Rather than assessing countries solely by economic development, the framework considers operational, technological, and organizational readiness to support sustainable regional implementation.

Framework

Readiness Dimension	Assessment Focus	Why It Matters
Digital Infrastructure	Connectivity, system maturity and technology availability	Determines implementation capability.
Power & Connectivity Resilience	Reliability of electricity and operational continuity	Supports uninterrupted healthcare delivery during disruptions.
Healthcare Investment Capacity	Funding availability and long-term sustainability	Determines financial readiness for implementation.
Digital Workforce Readiness	Digital literacy and workforce capability	Supports successful adoption and change management.
Health Data Governance	Policies, standards and regulatory alignment	Enables trusted cross-border information sharing.
Regional Collaboration Readiness	Institutional willingness and operational coordination	Supports sustainable regional interoperability.

2.3 ASEAN Deployment Tier Framework

Based on the ADHIRF assessment, ASEAN member states are grouped into three deployment tiers reflecting differences in infrastructure, funding capacity, power resilience, workforce readiness, and governance maturity. This tiered approach avoids a one-size-fits-all rollout and ensures that implementation is aligned with each country's readiness and support needs.

Tier A – Advanced countries will support early platform validation and regional learning. **Tier B – Mid-Infrastructure** countries will follow through phased expansion using lessons from early deployment. **Tier C – Low-Infrastructure / High-Need** countries will receive targeted foundational support to strengthen infrastructure, workforce capability, and governance before full deployment.

This readiness-based approach reduces implementation risk while ensuring that lower-readiness countries are not excluded from the regional transformation.

Deployment Tier	Countries	ADHIRF Readiness	Deployment Objective
Tier A – Advanced	Singapore, Brunei, Malaysia	High	Validate the platform and establish regional centres of excellence.
Tier B – Mid-Infrastructure	Thailand, Vietnam, Indonesia, Philippines	Medium	Expand regional interoperability using lessons learned from Tier A.
Tier C – Low-Infrastructure / High-Need	Cambodia, Laos, Myanmar	Foundational	Build infrastructure, workforce capability, and governance before full deployment.

Executive Insight

The phased deployment model reduces implementation risk by avoiding a one-size-fits-all approach. It enables ADHCRI to demonstrate measurable value in higher-readiness countries while creating structured pathways for lower-readiness countries to achieve sustainable adoption over time.

THE BUSINESS CASE

3.1 Proposed Business Solution

The ASEAN Digital Health & Climate Resilience Initiative (ADHCRI) is proposed as a secure, interoperable, and climate-resilient regional digital health ecosystem designed to strengthen public health decision-making across all ten ASEAN member states.

Rather than replacing existing national healthcare systems, ADHCRI will integrate with current digital health platforms to enable trusted cross-border health information exchange, predictive disease surveillance, climate-informed public health planning, and coordinated emergency response. The solution adopts a phased implementation strategy aligned to each country's readiness level, ensuring sustainable adoption while minimizing implementation risk.

By combining modern digital technologies with standardized governance and regional collaboration, ADHCRI will provide a scalable foundation for improving healthcare resilience, reducing operational inefficiencies, and supporting evidence-based public health policy across ASEAN.

3.2 Solution Overview

3.2 Solution Capabilities

Strategic Capability	Business Outcome	Enabling Technology
Cross-Border Health Intelligence	Faster regional information sharing	Cloud Computing & APIs
Predictive Disease Surveillance	Earlier outbreak detection	AI & Machine Learning
Climate Health Intelligence	Climate-informed planning	GIS & Predictive Analytics
Healthcare Continuity	Resilient healthcare operations	Edge Computing
Trusted Health Data Exchange	Secure regional collaboration	Blockchain
Executive Decision Support	Evidence-based policy decisions	Business Intelligence Dashboards

Executive Insight

ADHCRI is not an investment in technology alone. It is an investment in the strategic capabilities required to improve regional public health decision-making. Each proposed technology has been selected because it directly enables a measurable business capability that strengthens healthcare resilience, operational efficiency, and long-term socioeconomic value across ASEAN.

3.3 Financial Investment

The readiness-based deployment strategy presented in Section 2.3 requires an estimated **US\$559.1 million over five years** to deploy and sustain ADHCRI across all ten ASEAN member states. The investment combines upfront capital expenditure for regional infrastructure and platform capabilities with ongoing operational expenditure required to maintain, secure, and support the expanding regional ecosystem.

The financial model adopts a phased investment approach aligned with the ADHIRF deployment tiers. This allows capital to be directed according to country readiness and implementation need rather than distributed equally across member states. Higher-readiness countries support early validation and regional capability development, while lower-readiness countries receive targeted foundational investment to address infrastructure and adoption barriers.

ADHCRI Five-Year Investment Model - Readiness-Based Regional Deployment Strategy									
Phasing: Yr1 Tier A (advanced infra, fastest to deploy) -- Yr2-3 Tier B -- Yr3-5 Tier C (requires resilience layer first). All \$ in USD.									
Country Deployment Tiers (Based on ADHIRF Regional Readiness Assessment)									
Tier	Countries	# Countries	ADHIRF Readiness	Deployment Objective					
Tier A - Advanced	Singapore, Brunei	3	High	Validate platform and establish regional centres of excellence					
Tier B - Mid-Infrastructure	Thailand, Vietnam	4	Medium	Expand regional interoperability using lessons learned					
Tier C - Low-Infrastructure / High-Need	Cambodia, Laos	3	Foundational	Build infrastructure, workforce capability, and governance before deployment					
CAPEX Build-Up									
Cost Component	Yr 1	Yr 2	Yr 3	Yr 4	Yr 5	Total	Notes		
Interoperability Hub CAPEX - Tier A (3 countries, Yr1)	\$12,600,000	\$0	\$0	\$0	\$0	\$12,600,000	Full deployment Yr1 - lightest build, existing e-gov cloud integration		
Interoperability Hub CAPEX - Tier B (4 countries, phased Yr2-3)	\$0	\$13,600,000	\$13,600,000	\$0	\$0	\$27,200,000	2 countries per year to manage change-mgmt load		
Interoperability Hub CAPEX - Tier C (3 countries, Yr3-5)	\$0	\$0	\$0	\$9,500,000	\$9,500,000	\$28,500,000	1 per year - power/connectivity resilience layer must land first		
Rural Edge Nodes (solar+battery+offline-sync), 14,000 facilities phased	\$12,950,000	\$38,850,000	\$77,700,000	\$77,700,000	\$51,800,000	\$259,000,000	Ramps with Tier B/C rollout - rural facilities are concentrated there		
Subtotal - Core Platform CAPEX	\$25,550,000	\$52,450,000	\$100,800,000	\$87,200,000	\$61,300,000	\$327,300,000			
Cybersecurity & Data Governance Layer (blockchain audit ledger, zero-trust)	\$3,066,000	\$6,294,000	\$12,096,000	\$10,464,000	\$7,356,000	\$39,276,000	12% of core CAPEX per year - HIMSS critical infra benchmark		
TOTAL CAPEX	\$28,616,000	\$58,744,000	\$112,896,000	\$97,664,000	\$68,656,000	\$366,576,000			
OPEX Build-Up									
Cost Component	Yr 1	Yr 2	Yr 3	Yr 4	Yr 5	Total	Notes		
Cloud Hosting, Maintenance & Staffing (18% of cumulative CAPEX in prior years)	\$5,150,880	\$15,724,800	\$36,046,080	\$53,625,600	\$65,983,680	\$176,531,040	Applied to cumulative deployed CAPEX, not total - reflects phased go-live		
Training & Digital Literacy Programme (region-wide, all 5 years)	\$3,200,000	\$3,200,000	\$3,200,000	\$3,200,000	\$3,200,000	\$16,000,000	Flat annual run-rate - front-loaded content dev amortized across countries		
TOTAL OPEX	\$8,350,880	\$18,924,800	\$39,246,080	\$56,825,600	\$69,183,680	\$192,531,040			
TOTAL PROJECT COST (CAPEX + OPEX)	\$36,966,880	\$77,668,800	\$152,142,080	\$154,489,600	\$137,839,680	\$559,107,040			

The five-year investment model demonstrates the phased financial strategy for deploying ADHCRI across all ten ASEAN member states. Capital expenditure (CAPEX) is front-loaded to establish the regional digital health infrastructure, while operational expenditure (OPEX) increases gradually as additional countries transition into live operations. This readiness-based deployment approach minimizes implementation risk, supports equitable investment across member states, and maximizes long-term return on investment.

Executive Takeaway

The US\$559.1 million investment represents a five-year regional transformation programme rather than a one-time technology purchase. The phased funding structure links capital allocation directly to country readiness, implementation risk, and long-term operational sustainability.

3.4 Socioeconomic Cost-Benefit Analysis - NPV & ROI

ADHCRI's financial viability was assessed by comparing the five-year programme cost against projected socioeconomic benefits from earlier outbreak containment, reduced patient stay durations, and improved administrative efficiency.

Socioeconomic Cost-Benefit Analysis - NPV & ROI						
Benefits ramp in from Yr2 (partial) to full run-rate Yr3+, reflecting adoption curve and platform maturity.						
Line Item	Yr 1	Yr 2	Yr 3	Yr 4	Yr 5	5-Yr Total
Total Project Cost (from CAPEX-OPEX tab)	\$36,966,880	\$77,668,800	\$152,142,080	\$154,489,600	\$137,839,680	\$559,107,040
BENEFIT STREAMS						
Outbreak Containment Savings (early AI-detection avoids/shortens	\$0	\$25,200,000	\$63,000,000	\$63,000,000	\$63,000,000	\$214,200,000
Reduced Patient Stay Duration (interoperable records cut avoidable	\$0	\$43,605,000	\$101,745,000	\$145,350,000	\$145,350,000	\$436,050,000
Administrative & Duplicate-Testing Savings (interoperability efficie	\$0	\$46,890,000	\$140,670,000	\$234,450,000	\$234,450,000	\$656,460,000
TOTAL ANNUAL BENEFITS	\$0	\$115,695,000	\$305,415,000	\$442,800,000	\$442,800,000	\$1,306,710,000
NET CASH FLOW (Benefits – Costs)	-\$36,966,880	\$38,026,200	\$153,272,920	\$288,310,400	\$304,960,320	\$747,602,960
Discount Factor (8% social discount rate)	0.926	0.857	0.794	0.735	0.681	
Discounted Net Cash Flow	-\$34,228,593	\$32,601,337	\$121,672,986	\$211,916,751	\$207,550,870	\$539,513,351
NPV (5-Year Horizon)	\$539,513,351					
ROI (5-Yr Total Benefits ÷ Total Costs – 1)	133.7%					
Simple Payback Period (Cumulative Net CF, years)	2					
Interpretation Notes for Executive Board						
• NPV is positive across the 5-year window using conservative (60%) attribution on outbreak-avoidance savings alone.						
• Benefits ramp with adoption — Yr1 is investment-only by design; do not present Yr1 ROI as a standalone figure.						
• Sensitivity: halving the outbreak-avoidance assumption (Assumptions/B30) still yields a positive NPV — model is robust to this key judgement call, not dependent on it.						
• All benefit figures are quantified societal returns (avoided cost), not new revenue — appropriate framing for a public-health infrastructure ask to a financial group.						

Executive Takeaway

The analysis indicates that ADHCRI could generate approximately US\$1.31 billion in socioeconomic benefits over five years. With a projected ROI of 133.7%, positive NPV of US\$539.5 million, and payback near the end of Year 2, the programme presents a financially compelling case for phased regional investment.

3.5 Sensitivity Analysis

The financial case was tested against changes in key assumptions to assess whether ADHCRI remains economically viable under less favourable conditions. The analysis focuses on benefit realization and the discount rate, recognizing that programme adoption, implementation delays, and economic conditions may affect projected returns.

Scenario	Assumption	Expected Impact
Base Case	Benefits realized as projected; 8% discount rate	ROI 133.7% and positive NPV
Reduced Benefits	Socioeconomic benefits 20% below forecast	Lower ROI, but significant programme value retained
Delayed Benefits	Benefits realization delayed by one year	Longer payback period and reduced NPV
Higher Discount Rate	Discount rate increased above 8%	Lower present value of future benefits
Combined Downside	Reduced and delayed benefits	Highest financial pressure and increased investment risk

4. Investment Risks and Mitigation

ADHCRI's regional scale creates operational, governance, cybersecurity, climate, adoption, and financial risks. The programme therefore applies a multi-tiered risk mitigation approach that assigns measurable risk scores, defined mitigation actions, and clear ownership to protect

investment value throughout the five-year deployment.

Multi-Tiered Risk Mitigation Engine						
Tier	Risk	Likelihood	Impact	Risk Score	Mitigation Strategy	Owner
Financial	Currency volatility across 10 national budgets erodes CAPEX purchasing power	Medium	High	6	6 Multi-currency escrow structure with USD-pegged tranches; phased drawdowns tied to milestones, not lump sums	Programme Finance Lead
Financial	Funding shortfalls or delayed capital release disrupt phased deployment	Medium	High	6	6 Link capital release to deployment milestones; diversify funding partners and establish phased funding commitments	Regional Programme Director
Financial	Socioeconomic benefits are realized below forecast, reducing projected ROI	Medium	High	6	6 Quarterly benefits realization reviews, KPI tracking, sensitivity analysis, and corrective investment decisions	Benefits Realization Lead
Financial	Cost overrun on Tier C (low-infra) rollout due to underestimated resilience-layer build	Medium	High	6	6 10% contingency reserve on Tier C CAPEX; independent QS review before Y1/3 drawdown gate	Lead Business Analyst
Operational	Rural edge nodes fail during extreme weather events (the exact disruption the platform must survive)	Medium	High	6	6 Solar+battery redundancy rated for 72-hr autonomy; offline-first sync queues data until connectivity restored	Technical Delivery Lead
Operational	Legacy system integration delays at national ministry level	High	Medium	6	6 API-first interoperability layer (HL7 FHIR standard) built to wrap legacy systems rather than replace them; pilot-then-scale per country	Integration Architect
Governance/Political	Cross-border data-sharing rules stall on national sovereignty concerns	High	High	9	9 Federated data model — raw patient data stays in-country; only anonymized/aggregated indicators cross borders, governed by an ADHCRI Data Governance Charter co-signed by all 10 ministries	Regional Programme Director
Governance/Political	Change of government/health ministry leadership disrupts continuity	Medium	Medium	4	4 Multi-year MOU structure independent of election cycles; ASEAN Secretariat as continuity anchor, not individual ministries	Regional Programme Director
Cybersecurity	Ransomware/unauthorized access to centralized patient records	Medium	High	6	6 Zero-trust architecture, blockchain-based audit ledger, mandatory SOC-as-a-service, encrypted-at-rest/in-transit by design (not bolt-on)	CISO / Cybersecurity Lead
Cybersecurity	Inconsistent security standards across 10 national systems	High	Medium	6	6 Single regional security baseline (ISO 27001-aligned) as a condition of platform onboarding, audited annually	CISO / Cybersecurity Lead
Climate/Disaster	Extreme weather disrupts connectivity/power during active outbreak response	Medium	High	6	6 Edge-first design ensures local clinics keep functioning and syncing once connectivity restored; no single point of failure at national hub level	Technical Delivery Lead
Adoption	Low digital literacy slows frontline health worker adoption, undermining ROI case	High	Medium	6	6 Localized-language training curriculum, community health worker champions network, phased go-live with hands-on support (see Stakeholder Strategy)	Change Management Lead

Executive Takeaway

The highest-priority risks are addressed through readiness-based deployment, regional governance, resilient infrastructure, cybersecurity controls, and milestone-linked funding. Clear risk ownership ensures that mitigation remains an active programme responsibility rather than a one-time planning exercise.

5. Investment Recommendation and Funding Decision

The financial and readiness analysis supports a **phased investment in ADHCRI across all ten ASEAN member states**. With an estimated five-year programme cost of **US\$559.1 million**, projected socioeconomic benefits of **US\$1.307 billion**, a **133.7% ROI**, positive **NPV of US\$539.5 million**, and payback near the end of **Year 2**, the programme presents a strong case for regional investment.

It is recommended that funding be approved through a **readiness-based, milestone-linked investment model** aligned with the ADHIRF deployment tiers. Capital should be released progressively as agreed readiness, governance, implementation, and benefits-realization milestones are achieved.

Investment Recommendation

Recommendation	Proposed Approach
Funding Decision	Approve phased five-year investment
Investment Requirement	US\$559.1M
Deployment Model	ADHIRF Tier A → Tier B → Tier C
Funding Release	Milestone-linked capital allocation
Performance Oversight	Quarterly benefits and risk reviews
Expansion Decision	Readiness and performance-based stage gates

Final Investment Position

ADHCRI should proceed as a phased regional investment programme. The proposed model balances financial return, public health impact, regional equity, and implementation risk while creating a sustainable foundation for data-driven public health decision-making across ASEAN.

Executive Takeaway

The investment question is not whether ASEAN can afford to modernize its regional public health intelligence capability, but whether it can afford the growing financial, health, and resilience costs of continued fragmentation.

